Medical Consultation Report

Patient Information

Patient Name	Барахтенко Егор Андреевич
Gender	Male
Date of Birth	April, 24, 2008
Diagnosis	Refractory secondary acute myeloid leukemia

Evaluation of the patient's current disease status

The patient was diagnosed with myelodysplastic syndrome (MDS) in November 2024, with NRAS mutation and high WT1 expression. After initial treatment, the patient underwent the first allogeneic hematopoietic stem cell transplantation from an unrelated donor in April 2025 and achieved a brief remission. In July 2025, the disease transformed into secondary acute myeloid leukemia (sAML) with FLT3-ITD, WT1, and BAALC mutations/high expression. In September 2025, the patient received a second haploidentical transplantation from the father donor but experienced early relapse. Subsequent lines of targeted therapy, chemotherapy, and donor lymphocyte infusions (DLIs) were all ineffective. In April 2026, the patient received anti-CD33 CAR-NK cell therapy but showed rapid disease progression, with bone marrow blasts reaching 71%, MRD 51.2%, and peripheral blood blasts 35%. Currently, the patient is complicated by bilateral multilevel mixed pulmonary infection, acute kidney injury, suppurative otitis media, and pansinusitis, with a very poor performance status. This represents end-stage refractory leukemia, and all standard curative treatment options have been exhausted.

Doctor's Consultation

This patient has secondary acute myeloid leukemia transformed from MDS, accompanied by high-risk mutations such as FLT3-ITD, representing a subtype with extremely high malignancy and strong aggressiveness. Despite receiving multiple potent treatment regimens, including two transplants and CAR-NK cell therapy, the patient experienced rapid relapse, indicating that the tumor clone is highly drug-resistant and proliferatively active. In addition, the patient has concurrent renal impairment and severe pulmonary infection, with poor organ reserve and low treatment tolerance. This constitutes an extremely high-risk, rapidly progressing critical stage. A gentle yet precise individualized treatment approach must be adopted as soon as possible; otherwise, the patient's condition will continue to deteriorate rapidly.

This patient has extremely severe refractory AML that has relapsed after two transplants. The overall condition is critical, complicated by renal impairment and active pulmonary infection. No standard curative treatment options remain, but there is still an opportunity for individualized salvage therapy. First, it is necessary to clarify the patient's performance status, the most recent renal function, and the status of pulmonary infection control. If the patient's physical condition allows travel to the hospital, a systematic evaluation and comprehensive gene mutation testing will be performed. Based on the test results, the most appropriate and safest individualized targeted drugs will be selected. If necessary, antibody therapy may be combined to control the disease, alleviate symptoms, and extend survival as much as possible, while protecting organ function and managing the infection. It is recommended to conduct a Zoom online consultation as soon as possible to allow the physician to accurately assess the patient's real-time status, avoid delays in treatment, and prevent missing the critical window for intervention.

Treatment Plan

Phase I: Precise Re-assessment Upon Admission Assessment

1.Comprehensive assessment of performance status, symptoms, bleeding risk, and infection risk;

2.Re-examination of complete blood count (CBC), biochemistry, renal function, electrolytes, and glomerular filtration rate (GFR);

3.Chest CT, inflammatory markers, and etiological examination to evaluate the control of pulmonary infection;

4.Bone marrow aspiration, immunophenotyping, MRD monitoring, and comprehensive genetic mutation screening (full-coverage gene mutation testing);

5.Cardiac function assessment, coagulation function tests, and pre-transfusion-related laboratory tests.

Phase II: Salvage Core Therapy

1．Based on the results of genetic mutation testing, select the safest and most appropriate targeted therapy;

2.Based on immunophenotype and clinical needs, combine with antibody drugs for synergistic treatment when necessary;

3.Focus on protecting renal function, controlling pulmonary infection, and stabilizing organ function as core priorities;

4.Comprehensive supportive care throughout treatment: hematopoietic support, anti-infection therapy, fluid replacement, and symptomatic relief;

5.Regularly monitor treatment efficacy and safety indicators, and dynamically adjust the treatment regimen as needed.

Phase III: Subsequent Treatment

Timing: To be determined based on the response to Phase 2 treatment

Cost: To be separately evaluated according to the patient's condition, treatment response, and any complications

Develop a subsequent treatment plan based on the efficacy of Phase 2 treatment, disease control status, and recovery of organ function;

If necessary, provide consolidation/maintenance therapy, supportive and symptomatic care, and long-term infection control;

Dynamically monitor disease status and quality of life, and adjust the overall treatment strategy accordingly.

Estimated Treatment Duration and Cost (Based on an exchange rate of 1 USD = 7 RMB; final costs will be calculated based on the actual exchange rate upon admission. Hospitalization fees are settled in RMB)

Therapeutic procedure	**Estimated** Time **Duration**	**Items**	**Cost Estimation (RMB)**	**Cost Estimation (USD)**
Step 1. Assessment				
Pre-treatment assessment (special blood tests & image exams)	7 days	Genetic and molecular testing,Bone Marrow Aspiration exams (morphology, flow cytometry, gene mutation, chromosome test, bone marrow biopsy etc.） Lumbar Puncture (including CSF flow cytometry） Blood Test (including Biochemical Test, Infection-related Items) Thrombosis and bleeding assessment PET-CT, Lung-CT (optional), Echo, abdominal ultrasound, Brain MRI, ECG, Minimal Residual	105,000-140,000	15,000-20,000

		Disease, Pre-transplant screening		
Step 2. Salvage core therapy (Targeted therapy + immunomodulation + antibody therapy)				
(Targeted therapy + immunomodulation + antibody therapy)	2-4weeks	Targeted therapy Immunomodulatory therapy Targeted antibody therapy Supportive care Complication management	210,000-420,000	30,000-60,000
Total Hospitalization	3-5 weeks	Other cost including standard private room, nursing fee, consultations, treatments procedures, etc.	70,000	10,000
Total amount（including step1.2）	3-5 weeks	Above all	385,000-630,000	55,000-90,000
Step 3. Subsequent Treatment				
Develop a subsequent treatment plan based on the treatment efficacy of Phase 2, disease control status, and recovery of organ function; provide consolidation/maintenance therapy, supportive and symptomatic care, and long-term infection control if necessary; dynamically monitor disease status and quality of life, and adjust the overall treatment strategy accordingly.				
The above costs are estimated based on the patient's current physical condition. As the condition progresses, the treatment plan may change, and the costs may also vary.				

The cost includes:

（1）Single-room accommodation

（2）All necessary tests as per the treatment plan

（3）Medications, cell immunotherapy, and other treatment costs in accordance with the treatment plan

（4）Routine medical services, doctor consultations, examinations, laboratory tests, etc.

The payment doesn’t include:

(1) Food;

(2) Discharge medicine;

(3) Unexpected treatment and examination such as life-threatening infection, serious side effects

or ICU fees.

(4) Treatment for unrelated disease;

(5) Unexpected emergency management;

(6) Personal expenses such as phone calls, flight tickets.

Payment Terms:

Initial Deposit: A minimum initial deposit of 40,000 USD equivalent to 280,000 RMB is required upon admission to the hospital. This deposit will be applied towards the estimated total cost of treatment.

Final Payment: The remaining balance will be due upon completion of the treatment. Any additional costs incurred during the treatment process will be added to the final payment, and any unused funds from the initial deposit will be refunded.

Important considerations before treatment：

Given that the patient has end-stage refractory AML with relapse after two transplants, failure of multiple lines of therapy, complicated by renal impairment and pulmonary infection, the condition is extremely critical. Every day of delay may result in the loss of any treatment opportunity. Renal function and pulmonary infection control are the key determinants of whether treatment is feasible. Please provide the most recent results on renal function, pulmonary infection status, and performance status as soon as possible, and arrange an immediate Zoom online consultation for direct evaluation by the attending physician. This will allow us to determine concurrently whether the patient can be admitted and whether appropriate medications are available, enabling a rapid assessment of whether any treatment opportunity still exists and avoiding any delay in this last critical window for intervention.

Given the complexity of your condition, we strongly recommend scheduling a one-on-one personalized online consultation with Dr. Zhao, our Director of the Department of Hematology: Zhao Defeng. He has extensive experience in treating Refractory secondary acute myeloid leukemia.

This exclusive, appointment-based consultation will provide you and your family with:

（1）A detailed assessment of your current condition

（2）Preliminary treatment guidance tailored to your needs

（3）An opportunity to ask critical questions directly to a senior specialist

As part of our commitment to international patients, this consultation is offered at no charge, and spaces are limited due to Dr. Zhao’s clinical schedule. We encourage you to confirm your interest early to ensure availability. Interpretation in English is available free of charge. For other languages, professional interpretation can be arranged upon request (USD 100 per 30 minutes), or you are welcome to bring your own interpreter.

We strongly encourage proceeding with the remote consultation as soon as possible to avoid delay in treatment planning and cell collection timeline

Important Notes

（1） The above preliminary treatment plan is based on the information provided by the patient and/or their representative. Upon hospital admission, a comprehensive re-evaluation will be conducted. If there are significant discrepancies between actual condition and previously submitted history or test results, the treatment plan and costs may be adjusted accordingly after discussion with the patient and/or legal guardian.

（2） One week prior to admission, it is necessary to provide recent external laboratory test reports and the patient's general condition, such as:

A recent week's complete blood count (CBC) report to assess for bone marrow suppression or infection risk.
Liver and kidney function reports, as well as cardiac function reports, to ensure the patient can tolerate chemotherapy.

Infection control status, to confirm whether the patient has a severe infection.

Physical condition assessment, especially to check for symptoms of GVHD (graft-versus-host disease).

Blood type and RH blood type reports, in preparation for using donor cells during treatment.

We understand that seeking medical treatment abroad is a physical and emotional challenge for both patients and their families. Our hospital provides multilingual support, cultural sensitivity, personalized meals, assistance with accommodation and transportation, and family companion services to create a safe and welcoming environment. From airport pickup and admission to recovery and follow-up, our international patient service team will accompany you throughout the entire process. Rest assured, we are committed to offering professional guidance and compassionate care to ensure your treatment journey is clear, reassuring, and well-managed.

Director of the Department of Hematology: Zhao Defeng

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Date: May 12, 2026